



YMNOTES MEDICAL

Obstetric Cases

Obstetrics · Case Scenario – Abortion

CASE SCENARIO SERIES · 2026

About this edition

SOURCE

Redesigned from the YMnotes “Obstetric cases” handout (6 scanned pages): Case Scenario – Abortion.

WHAT CHANGED

Presentation only. The medical text is reproduced from the original book: typography, hierarchy, page layout, tables, figures and navigation were rebuilt; the wording, numbers, doses, units, thresholds, classifications and recommendations were not.

WHAT WAS CORRECTED

Any evidenced change to supplied wording is identified in the correction register with its original form. Uncertain source readings remain explicitly unresolved.

EDITORIAL ADDITIONS

Chapter grouping, navigation labels, figure numbers and source-page references were added by YMnotes for navigation. Editorial notes are shown in a distinct panel so they can never be mistaken for the source book's own words.

FIDELITY

The medicine in this edition is the medicine in the original book. Nothing has been added, removed, reworded or summarised; only the presentation is new. Where the original was demonstrably defective, the repair is marked on the page and listed in the editorial corrections.

REPAIRED SOURCE DEFECT

Where the supplied source cannot establish a character or reading, uncertainty is retained. A source restoration must be evidenced by the original material, not inferred from medical knowledge.

OTHER CORRECTIONS

Typographical defects in the source — misspelled drug names, malformed medical terms, lost word spaces — were corrected where the evidence was unambiguous, and marked where the correction changes a word. Nothing was rewritten for style, and no medical statement was modernised: where the source is dated rather than wrong, it stands as written.

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How to read this book

CHAPTER OPENER

Each chapter opens on its own page with the chapter number and the original book's own title for that material.

TOPIC BAND

A tinted band with a heavy left rule marks a major topic. The running header repeats the current topic on every page.

CONDITION

A named disease, drug or entity sits under a lighter rule directly beneath its topic.

RECURRING SECTIONS

Definition, Causes, Clinical Picture, Investigations, Treatment and Complications carry a small accent marker. Treatment runs wide and dark; complications and contraindications run red; investigations run blue.

BULLET DEPTH

A filled diamond is a first-level point, a filled square a second, a short dash a third and a hollow circle a fourth. Shape carries the depth — not size or colour — so the nesting survives greyscale printing and reads at arm's length.

EDITORIAL NOTE

A dashed violet panel is written by YMnotes, not by the original book. It explains organisation only and adds no medical content.

EDITORIAL CORRECTION

A pale amber word, symbol or number is one YMnotes verified and corrected or restored from the supplied source. The original reading is recorded for every one of them.

SOURCE PAGES

Figure captions cite the page of the original book they came from, so any passage can be traced back to the source.

CHAPTER 01

Case Scenario - Abortion

IN THIS CHAPTER

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01**YMnotes**



Personal History

- ◆ **الطبيب:** اسمك إيه؟ وعندك كام سنة؟
- **المریضة:** اسمي سارة محمود عبد الرحمن، عندي 19 سنة.
- ◆ **الطبيب:** متجوزة بقالك قد إيه؟
- **المریضة:** بقالنا حوالي 6 شهور.
- ◆ **الطبيب:** ساكنة فين؟
- **المریضة:** في عند فودافون.
- ◆ **الطبيب:** بتشريبي سجائر أو أي حاجة ممكن تأثر على صحتك، بتربي قطط؟
- **المریضة:** لأ خالص.
- ◆ **الطبيب:** جوزك اسمه ايه، عنده كام سنه، بيشغل ايه، طيب جوزك بيشرب سجائر أو أي حاجة ممكن تكون بتأثر على صحته؟
- **المریضة:** اسمه سالم عوض خيرالله عند 24 سنه، بيشغل سواق نقل، بيشرب سجائر كتير، تقريبا علبة في اليوم.

Complain

- ◆ **الطبيب:** طيب، إيه اللي تعبك وجايبك النهارده؟
- **المریضة:** من امبارح بالليل لقيت نقط دم بسيطة على الهدوم، لونها أحمر فاتح، بس مفيش ألم ولا حاجة.

History of present pregnancy

- ◆ **الطبيب:** اخر دوره كانت امتي؟
- **المریضة:** اخر دوره كانت 25 اغسطس 2026.
- ◆ **الطبيب:** عرفتني منين انك حامل؟
- **المریضة:** لما الدورة اتاخرت عملت الاختبار اللي ف البيت.
- ◆ **الطبيب:** الدم كتير ولا مجرد نقط؟
- **المریضة:** لأ مجرد نقط خفيفة، ومفيش تكتلات ولا قطع دم.
- ◆ **الطبيب:** في وجع في بطنك أو ضهرك؟
- **المریضة:** لأ خالص.
- ◆ **الطبيب:** في دوخة، ضربات قلبك سريعة، أو ضيق نفس؟
- **المریضة:** لأ الحمد لله.



♦ **الطبيب:** في إفرازات أو ريحة مش طبيعية من المهبل؟

– **المريضة:** لأ، مفيش أي إفرازات.

♦ **الطبيب:** طيب تابعتي الحمل فين، عملتي التحاليل اللي في اول الحمل، بتاخدي ادويه ايه للحمل؟

– **المريضة:** انا ملحقتش، متابعتش الحمل وكنت ها اروح للمركز الأسبوع اللي جاي.

♦ **الطبيب:** طب عندك أي اعراض اخري؟

– **المريضة:** لأ.

♦ **الطبيب:** عندك مشاكل في القلب او التنفس، بتاخدي أي علاج لاي امراض اخري؟

– **المريضة:** عندي السكر وباخد انسولين بس مش متضط.

♦ **الطبيب:** طب بتدخلي الحمام كتير، بتشربي ميه كتير، بتجوعي كتير؟

– **المريضة:** لأ.

♦ **الطبيب:** عملتي أي تحاليل او اشاعات او بتاخدي أي علاجات اخري؟

– **المريضة:** لأ.

Menstrual History

♦ **الطبيب:** آخر دورة كانت امتي؟

– **المريضة:** أول يوم فيها كان 25 أغسطس 2026.

♦ **الطبيب:** والدورة قبل كده كانت منتظمة، كنتي بتاخدي أي ادويه فيها هرمونات او أقراص منع حمل، متأكدة من تاريخ الدورة؟

– **المريضة:** دورتي مكنتش منتظمة، بس كانت بتيجي كل 28 يوم تقريبًا.

♦ **الطبيب:** كان في اعراض مصاحبه لنزول الدورة، كان في اعراض تبويض زي الم في نص الشهر او نقط دم بسيطة؟

– **المريضة:** لأ؟

Obstetric history – "obstetric code"

♦ **الطبيب:** حملتي كام مره قبل كده، خلفتي قبل كده، اجهضتي قبل كده، لو في حمل: كان تاريخه امتي –

ولدتني امتي في الحمل – ولدتي فين – ولد ولا بنت – ولدتي ازاي – كان وزن الببي كام – رضعتيه ولا لأ –

حصل مضاعفات في الولادة.... لو في اجهاض: اجهضتي امتي – اجهضتي في الشهر الكام من الحمل – دخلتي العمليات ولا لأ – حصل نزيف او مشاكل بعده؟

– **المريضة:** لأ، ده اول حمل ليا.

Past History "Medical & Surgical"

♦ **الطبيب:** عندك أمراض مزمنة؟

– **المريضة:** أيوه، عندي سكر من وأنا صغيرة.



- ◆ **الطبيب:** بتاخدي علاج إيه؟
- **المريضة:** باخد أنسولين، بس بصراحة السكر مش متظبط أوي.
- ◆ **الطبيب:** بتتابعي السكر بانتظام؟
- **المريضة:** لأ مش دايماً.
- ◆ **الطبيب:** في أعراض زي عطش جامد أو تبول كثير أو جوع زيادة؟
- **المريضة:** لأ، مش حاسة بكده.
- ◆ **الطبيب:** خدت أي أدوية أو فيتامينات من أول الحمل؟
- **المريضة:** لأ، لسه ماخدتش حاجة.
- ◆ **الطبيب:** عملتي أي عمليات قبل كده، نقلوا لك دم قبل كده، عندك حساسيه من اي دوا؟
- **المريضة:** لأ.

Contraceptive History

- ◆ **الطبيب:** استخدمتي أي موانع للحمل قبل كده؟
- **المريضة:** لأ.

Family History

- ◆ **الطبيب:** بابا او ماما عندهم ضغط او قلب او سكر؟
- **المريضة:** ايوه بابا وماما عندهم سكر وبابا عنده كمان الضغط.
- ◆ **الطبيب:** انتي وجوزك قرايب، حد في العائلة عنده أطفال فيهم عيوب خلقيه او تشوهات؟
- **المريضة:** لأ.

Discussion

Lecturer: Summarize the main complaint of this patient?

Candidate: Yes, doctor. The patient is 18 years old, in her first pregnancy, presenting with mild spotting of blood without pain, and her blood sugar is poorly controlled as she is an insulin-dependent diabetic.

Lecturer: What is your **provisional diagnosis**?

Candidate: My provisional diagnosis is **threatened abortion**.





Lecturer: What are the **points in favor** of this diagnosis?

Candidate:

- ◆ She is pregnant with a missed period and positive pregnancy test.
- ◆ She has mild vaginal spotting, not associated with pain or passage of clots.
- ◆ There are no systemic symptoms like dizziness or shortness of breath.
- ◆ No abdominal pain, suggesting the cervix may still be closed.

Lecturer: Very good. What are your **differential diagnoses**?

Candidate:

1. **Threatened abortion** – most likely.
2. **Inevitable or incomplete abortion** – if pain or tissue passage occurs.
3. **Ectopic pregnancy** – must be excluded if pain develops or ultrasound shows empty uterus.
4. **Molar pregnancy** – if β -hCG is excessively high or ultrasound shows no viable embryo.
5. **Local cervical cause** – like cervical erosion, polyp, or postcoital bleeding.

Lecturer: How will you **confirm the final diagnosis**?

Candidate:

1. **Transvaginal ultrasound** to check for an intrauterine gestational sac, fetal viability, and any subchorionic hematoma.
2. **Serial quantitative β -hCG levels** to assess normal rise of the hormone.
3. **Speculum examination** to ensure the cervix is closed and exclude local lesions.

Lecturer: What **investigations** would you request for this patient?

Candidate:

- ◆ **Pelvic ultrasound** for fetal viability.
- ◆ **CBC** to assess hemoglobin and general condition.
- ◆ **Blood sugar profile** (FBS, PPBS, HbA1c) to evaluate diabetic control.
- ◆ **Blood group and Rh typing.**
- ◆ **Urinalysis** for sugar and infection.
- ◆ **β -hCG** quantitative test.



Lecturer: How would you **manage** this case if it is threatened abortion?

Candidate:

- ◆ **Reassurance and rest:** Advise the patient to rest and avoid heavy work or sexual intercourse temporarily.
- ◆ **Control of diabetes:** Strict blood sugar control with adjustment of insulin dose in collaboration with an internal medicine specialist.
- ◆ **Supportive therapy:** Folic acid 400 µg daily, and adequate hydration and nutrition.
- ◆ **Observation:** Monitor for increase in bleeding or onset of pain.
- ◆ **Progesterone support:** May be prescribed if indicated after confirming intrauterine pregnancy.
- ◆ **Follow-up:** Repeat ultrasound after one week to assess fetal viability.

Lecturer: If the **bleeding increases or pain develops**, what would you suspect?

Candidate: I would suspect the progression to **inevitable or incomplete abortion**, and I would reassess the patient with ultrasound and possibly prepare for uterine evacuation if the miscarriage becomes complete or incomplete.

Lecturer: How does **poor diabetic control** affect pregnancy outcome?

Candidate:

- ◆ It increases the risk of spontaneous abortion.
- ◆ It may cause congenital malformations, especially neural tube and cardiac defects.
- ◆ It increases risk of macrosomia, preeclampsia, polyhydramnios, and perinatal complications.

Lecturer: what is the **prognosis** in this case?

Candidate:

If the bleeding remains mild and the fetus is viable with good diabetic control achieved, the prognosis is good and the pregnancy can continue normally. However, if diabetic control remains poor, the risk of miscarriage and fetal anomalies increases significantly.



Summary Table – Pregnant Woman with Mild Vaginal Spotting

Item	Details
Patient Profile	18-year-old, primigravida, insulin-dependent diabetic with poor control
Main Complaint	Mild vaginal spotting without pain, no clots, no systemic symptoms
Provisional Diagnosis	Threatened abortion
Differential Diagnoses	- Inevitable / Incomplete abortion - Ectopic pregnancy - Molar pregnancy - Cervical lesion or infection
Confirmatory Steps	- Transvaginal ultrasound → fetal viability, location of sac - Quantitative β-hCG → serial rise - Speculum exam → closed cervix, exclude local cause
Investigations	- CBC - Blood sugar profile (FBS, PPBS, HbA1c) - Urinalysis - Blood group and Rh typing - Pelvic ultrasound - Quantitative β-hCG
Management Plan	- Reassurance and bed rest - Avoid intercourse and exertion - Strict diabetic control with insulin adjustment - Folic acid 400 μg daily - Progesterone support if indicated - Follow-up ultrasound after 1 week
Prognosis	- Good if bleeding subsides and diabetes is well controlled - Poor control increases risk of miscarriage and fetal anomalies



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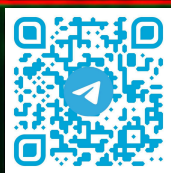
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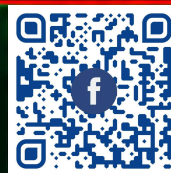
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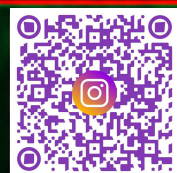
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